

Department of Health and Human Services

Attention: HHS Health Sector AI RFI (RIN 0955-AA13)

14 February 2026

Subject: Public Comment on Accelerating AI Adoption in Clinical Care

To the Department of Health and Human Services,

As a junior pre-medical student at Stanford University, my perspective on the integration of Artificial Intelligence (AI) into clinical care is shaped by distinct but complementary experiences: extensive shadowing of physicians across various clinical settings and my own encounters navigating the healthcare system as a patient. From the provider side, I have consistently witnessed physicians dedicating a substantial portion of their valuable time to administrative tasks, particularly documentation and navigating complex, often unintuitive, electronic health record (EHR) systems. From the patient side, I have experienced firsthand the long wait times, fragmented communication, and inefficiencies that define much of the modern healthcare experience. It is from this dual vantage point that I offer this comment, emphasizing the strategic deployment of AI to alleviate these burdens, enhance provider productivity, and ultimately strengthen the patient-doctor relationship.

One of the most immediate and impactful applications of AI in clinical care lies in the automation of documentation. My shadowing experiences have frequently shown doctors spending hours on "charting," often extending beyond their scheduled clinical shifts. This administrative overhead is a critical contributor to physician burnout and directly reduces the time available for meaningful patient engagement. The widespread adoption of AI-powered ambient listening tools, capable of recording and summarizing patient encounters into detailed clinical notes, is essential. By integrating such technologies into the clinical workflow, HHS can help redirect the physician's attention back to the patient, rather than the computer screen. Furthermore, by reducing the time required for each patient encounter through automated documentation, AI can effectively augment the capacity of our existing physician workforce, thereby alleviating the growing national doctor shortage without compromising the quality of care.

The U.S. healthcare system also grapples with exceptionally high administrative costs when compared to international counterparts, such as Taiwan. A considerable portion of this financial burden originates from intricate

billing and coding requirements. AI presents a significant opportunity to streamline these processes. Specifically, AI should be leveraged to automate administrative billing and insurance claims processing. This would not only lead to a substantial reduction in administrative expenditures but also minimize errors that frequently result in claim denials and delays in patient care. To prevent the fragmentation that characterized the early adoption of EHRs, HHS must proactively establish clear standards for the use of AI in billing and administrative tasks. Without a unified framework for these AI tools, there is a considerable risk of creating new layers of incompatibility, which would ultimately increase rather than decrease costs.

A critical barrier to the broader adoption of AI in healthcare is the substantial "switching cost" associated with transitioning away from dominant, legacy EHR systems like Epic. While these systems are often criticized for being slow and unintuitive, they form the operational backbone of most hospital systems. It is my belief that a more realistic and efficient approach involves the government partnering with these established EHR providers, rather than expecting nascent startups to completely disrupt the market. HHS should implement incentives designed to encourage legacy providers to modernize their systems, transforming them into streamlined, AI-powered platforms. My observations confirm that current EHR interfaces can be taxing and inefficient for providers. Therefore, government incentives should specifically target the integration of AI-driven, intuitive user interfaces within existing EHRs to facilitate rapid and widespread adoption. This approach would allow for mass implementation at an accelerated pace, leading to improved patient-doctor interaction time and helping to address the current shortage and inefficiencies within our physician population.

Beyond administrative efficiency, AI holds immense potential to improve health equity and access, particularly for underserved and rural populations where healthcare resources are strained. AI can serve as a crucial first-line triage tool, helping patients assess the urgency of their symptoms. For instance, an AI-powered initial screening could help a patient determine whether their condition warrants an immediate, and often hours-long, drive to an emergency room or if the issue can be safely addressed through a scheduled telehealth visit. By providing this initial guidance, AI can reduce the burden on over-capacity emergency departments and ensure that limited healthcare resources are allocated to the most pressing cases.

Furthermore, AI can revolutionize diagnostic processes by enabling the multi-purpose use of medical scans. Currently, imaging studies like MRI or CT scans are typically ordered to investigate a single condition, representing a one-dimensional layer of analysis. Research has shown that AI models can be trained to analyze a single scan for a

multitude of different conditions simultaneously. HHS should support a future where one scan can yield a broad range of preliminary diagnoses for a physician to confirm, rather than the current paradigm of one scan for one issue. This would not only be more efficient but would also increase the likelihood of catching incidental findings and other potential issues that might otherwise be missed.

Finally, to ensure the successful and safe integration of these tools, AI must become a core component of clinical training. As a future medical student, I believe it is imperative that the next generation of physicians is taught not only the fundamentals of medicine but also how to work effectively with AI. This training must go beyond basic use and include a deep understanding of AI's limitations, including its potential to "hallucinate" or produce convincingly incorrect answers. Physicians should be trained to prompt AI systems skillfully to reduce error rates and to critically parse AI-generated information. The goal should not be to wait for a completely error-free AI, which may never be realized, but to train clinicians to use AI as a powerful, imperfect tool. This approach, where the human remains in the loop and is accountable for the final judgment, will foster trust and ensure that the combination is greater than the sum of its parts.

Thank you for the opportunity to provide this perspective.

Sincerely,

Ryan Kern

Stanford University, Class of 2027

